

Category: ADVOCATE

RN / ASST. HEAD NURSE PRACTICE LEADER Skills care planning, Case Management, Home Health, Hospice, Infection control, injections, Nurse Manager, Oncology, scheduling, staff development, Trauma, triage, tutoring, Urology Experience 09/2010 to 12/2011 Company Name Hired as ADON (Assistant Director of Nursing) for an 85 to 90 bed long term care of Mentally and Physically disabled children from 2 years to upper 20's with a few older individuals. My job there included staffing of all the nurses monthly, setting up transportation for all outside MD office visits of the residents, monitoring the 3 nursing units of day to day care and documentation of the residents, taking call as needed and twice a month being MOD for the entire building ,I conducted in-services for the nursing staff and answered all pages during the day with questions or to oversee any critical changes of residents and helped with decisions to transfer out to a hospital after speaking with the MD.Helped with staff nursing as needed. Secured a Home Health position at ResCare located in Oak Park Illinois in 2015. I made home visits overseeing clients overall health. Did basic assessments and listened for any new complaints or change of status. Would call clients doctor if needed or to inform him of a change in status. Gave injections/ infusions as needed and participated in one clinical trial where all aspects of visit were timed and documented as per protocol. I am still registered with this agency but they have very few clients in need of an RN visit. Most of their clients only need part time CAN visits. Secured an RN position in July 2016 at Best Home Health where I did home visits part time. The jobs that were available for part time were minimal so took myself off their roster. In the fall of 2016 worked for Maxim Home Health asking for part time for Infusion cases. Again I was promised infusion cases but what were available Were ones 40 miles away and they didn't cover milage. I still am on their roster but rarely called. Currently looking for Part Time RN on line or going into a facility not more than 30 minute drive to do tutoring or online triage or willing to learn Case Management or PT Oncology/Hospice Additional professional activities Was OCN certified in past Was Med / Surg certified in past and plan to recertify in future. Was team nurse of North Shore Youth Baseball League for 4 years References Dr. Emil Totonchi MD. Advocate Illinois Masonic Med. Center** Dr. Michael Friedman MD. Advocate Illinois Masonic Med. Center** Beverly Bohus RN, Nurse Manager Advocate Illinois Masonic Med. Center** Lorel Hedges Nurse Practitioner. Advocate Illinois Masonic Med. Center now works in Tennessee Uma Kelkar Director of Rehab Manor Care of Northbrook Jean Gavina Director of MDS, Manor Care of Northbrook Elma Ramos RN , Evening Nursing Supervisor, Manor Care of Northbrook Contact information available upon request. 06/2009 to 07/2010 Company Name Started out as a Supervisor but was promoted to DCD (Director of Care Delivery) managing a 40 bed Locked Dementia Unit and later a 60 bed Long Term Care Unit and recently back to the Dementia Unit along with the Long Term Care Unit. Duties included: Leading the nursing unit, setting priorities, staff development ,upholding Corporate Standards, communicating through existing information systems, and material management, Clinical Management: Infection control, QA, understanding Professional issues, valuing every employee, supporting and leading company policies, attending interdisciplinary meetings twice daily, weekly and monthly meetings such as Medicare meetings, investigation of all falls and skin injuries, and new admissions care planning. Monthly Staff Meetings. Assists with hiring and disciplinary actions. Resigned July 2010 Want to take some vacation time, and to find a Long Term Care Management position somewhat closer to my home. 01/1977 to 01/2009 RN / Asst. Head Nurse Practice Leader Company Name ■ City , State Responsible for management of Unit on 24 hr basis including staffing, scheduling, and payroll. Monitor staff of RN's, NCT's, and Unit Clerks for daily assignments on going learning needs, and occasionally fill staff RN voids. Reported to Nurse Manager on daily basis for any additional tasks and to keep her current to Units activities and problems. Helped Nurse Manager in interview process and in both disciplinary activity and in employee terminations .Sought out and scheduled in-services for staff and made sure all staff attended required hospital mandates yearly and when required. 01/1977 to 01/2009 Company Name ■ City , State Started nursing career as RN on general Med / Surg Unit for 2 years. Recruited by MD Chairman of Oncology to be part of staff to open 1st Oncology inpatient Unit. Promoted also to Assistant Head Nurse. Worked there for next 20 years. Earned OCN certificate. Helped instruct all RNs on unit to become certified to mix Chemo drugs via Laminar Flow Hood and to administer drugs safely and monitor for side effects and manage such symptoms. Beside management duties was also responsible for taking a patient load on many days during the week. In 1999 changed to Surgical / 23hr Observation Unit at Illinois

Masonic Med Center as Staff RN. After approximately 4 years promoted to Practice Leader,(equal to Asst. Head RN) Duties expanded as Computers installed to all Units. Responsible for computer proficiency and scheduling staff to Computer Classes and monitoring their progress in daily charting on patients. Attended workshops to learn computer scheduling and payroll duties. Attended 2 day seminar for PICC LINE insertion and management. At one point headed the PICC LINE program for all the PICC RNs and kept the records for all PICCs placed in patients and any complications and later problems, Became Nationally certified for Med / Surg Nursing after taking exam and passing. Types of Surgical patients seen on thisUnit include Gyne, Urology, General Surgical, Trauma, Post SICU Head and Neck, General ENT patients and Fetal Demises less than 20 weeks. Due to the number of private rooms on the unit we also would take all types of Isolation including R/O TB into our Negative Air Flow room. Many of the patients were short stay anywhere from overnight to 2-3 days. Some of the patients as overflow Medicals, Isolation, and the more extensive Head and Neck Surgical patients would stay 4-5 days or more. As a result this Unit was fast paced. Many days up to half the unit would be discharged and within 1-2 shifts all the beds would be filled with new patients. My main job on a daily basis was to manage placing patients correctly and to oversee assignment of competent staff to the types of patients coming in. I made frequent rounds to assure pt satisfaction and to quickly aid in preventing and problems from escalating. I had to monitor staff learning needs and try to set up needed review or Unit based in-service and also try to send staff to outside learning seminars as budget allowed. On daily basis was responsible for checking" State of the Unit " printed daily and included all the areas of staff charting that was missing or incorrect. I would make sure all shifts of staff were aware of missing documentation and have them correct such or help them with correction as needed. Attended staffing daily to make sure last minute changes done and next two shifts had adequate staffing. Education and Training 1976 RN Diploma Illinois Masonic School of Nursing ■ City , State 1974 Northeastern Illinois University ■ City , State 1973 Western Illinois University ■ City , State Additional Information Awards received Dorothy Rome Nominee in 1999 (Highest yearly award for Experienced RNs) Awarded many "Applause Awards'" during the years at Masonic. (A nominated award for Excellence) Asked to Head up the PICC Line Program until turned over to Interventional Radiology All of the above from Advocate Illinois Masonic Medical Center.